

Health Care

by Sophia



WHAT'S COVERED

In this lesson, you will learn about health care in the United States in terms of business and ethics. In particular, you will learn about:

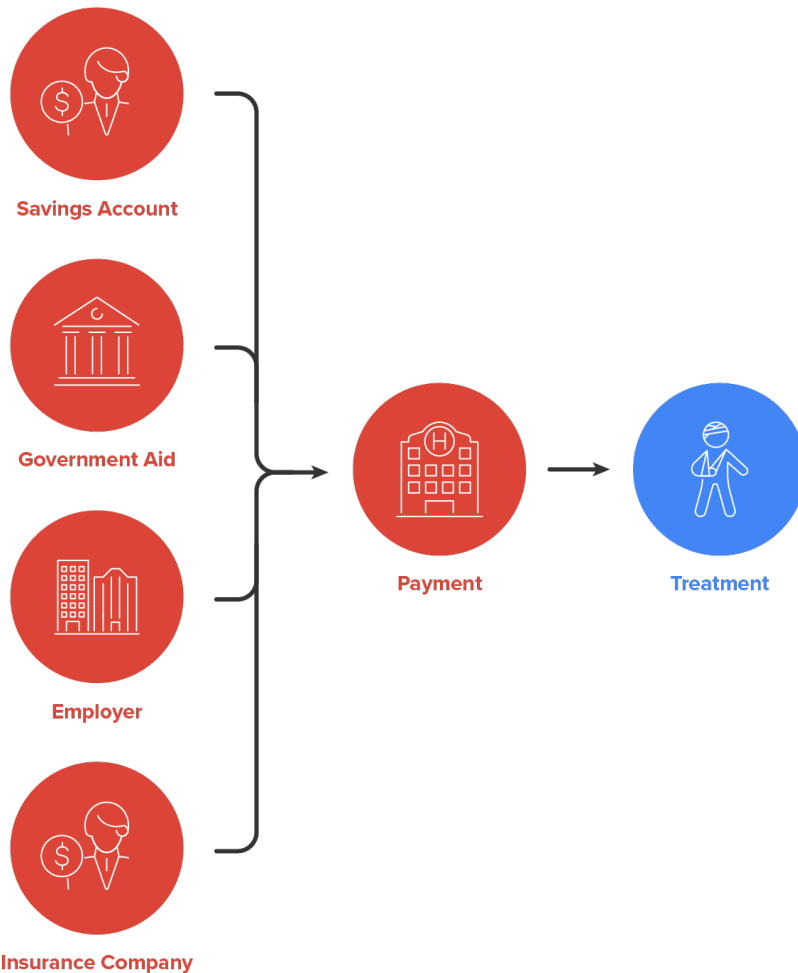
1. Single-Payer and Multi-Payer Health Care

Private health care in the United States has historically been of high quality and readily available, but only for those who could afford it. This model for rationing health care services is rare in the developed world and stands in dramatic contrast to the provision of health care in other industrialized economies. For those who provide health care and administer the health care system, balancing the quality of, access to, and cost of medical care is an ethical dilemma in which they must continually engage.

Typically, in the United States, medical services have been dispensed through a **multi-payer health care system**, in which the patient and others, such as an employer and a private health insurance company, all contribute to pay for the patient's care. Germany, France, and Japan also have multi-payer systems. In a **single-payer health care system** such as those in the United Kingdom and Canada, national tax revenues pay the largest portion of citizens' medical care, and the government is the sole payer compensating those who provide that care. Since a single payer system makes health care available to everyone, it is also called **universal health care**.

Both single- and multi-payer systems help reduce costs for patients, employers, and insurers; both, especially single-payer, are also heavily dependent on taxes apportioned across employers and the country's population. In a single-payer system, however, because payment for health care is coordinated and dispensed by the government, almost no one lacks access to medical services. Furthermore, because the government does not have to also yield a profit, health care is provided at lower cost to citizens, even with higher taxes.

Multi-Payer Health Care	Single-Payer Health Care



Funding for health care comes from several sources, and even people with health insurance usually have to pay some out-of-pocket expenses or rely on subsidies like Medicare and Medicaid.



All expenses are paid directly from the government to the health care provider.

Many reasons exist for the predominance of the multi-payer system in the United States. Chief among these is the U.S. tradition that doctors' services and hospital care are privatized and run for profit. Along with the profit motive, the fact that providers are compensated at a higher average rate than their peers abroad ensures that health care is more expensive in the United States than in most other nations.

Health care is the biggest industry in the U.S, generating over \$800 billion dollars a year and providing 22 million jobs. Two closely related industries—health insurance and pharmaceuticals—are also near the top of the list of biggest industries by revenue. The United States also has more health care professionals per citizen than most other countries, and more medical centers and clinics. One positive result is that the wait for most elective medical procedures is often shorter than in other countries, and travel time to a nearby medical facility is often less.

Still, paying for health care remains one of the most controversial topics in the United States, and many question what it is that Americans gain from the current system to balance the cost. As an exhaustive study from The Commonwealth Fund asserted, "the United States spends far more on health care than other high-income countries... yet the U.S. population has poorer health than other countries."

IN CONTEXT

While hospitals will not turn people away in urgent need of care, they can saddle them with debt. In a series of stories for National Public Radio, Noam Levy reveals that 100 million adults in the U.S.—40% of adults—have medical debt (“Sick and Struggling to Pay”). Many of them do have insurance through work, but the copays (patient responsibility) for hospital stays can be up to 50% and add up fast. Levy also reports that hospitals continue to reap high profits, often in the areas with the highest debt (“Some Hospitals Rake In High Profits”). Health care is unlike any other product because people have no option to *not* buy it when they need it. If hospitals are not refusing service, there is still an ethical question of maximizing profits while forcing people into ruinous debt.

Besides its inefficiencies, the state of U.S. health care raises challenging ethical issues for professionals in the field and for patients as well. What happens if many poorer people cannot afford health care? Should doctors treat them anyway? While they are required to treat anyone in immediate need of medical attention, there is no such requirement for people with chronic illnesses.

Who is qualified to receive subsidized (insured) health care? In the absence of universal health care, can the United States truly boast of being the richest nation on Earth? Some argue that universal coverage is itself unethical, forcing people with money to pay more than they spend so other people can get coverage. Even for supporters of universal coverage, there is the question of how to minimize the impact on the U.S. economy. A single-payer system would obviate the need for health insurance, put hundreds of thousands of people out of work, and evaporate many investments. With the government being, in effect, the sole customers of health care, they would also inevitably enforce providers' lower costs, reducing salaries of medical professionals. Most importantly, politically, is that such a change would be contrary to the idea of free market capitalism. For this reason, the American Medical Association has long been opposed to any sort of state-sponsored insurance.



THINK ABOUT IT

If universal health coverage did pass, what would the government owe investors in health insurance companies? What would they owe the employees at health insurance companies? What would they owe medical providers in terms of compensation?



TERMS TO KNOW

Multi-Payer Health Care System

Systems of health care where the costs of health care are paid by a combination of patients, employers, insurance companies, government, and private charities.

Single-Payer Health Care System (also called Universal Health Care)

Systems of health care where all or most of the costs of health care are managed by the government.

2. Pharmaceuticals

Not only is medical care enormously expensive in the United States, but so are prescription drugs. According to William B. Schultz, an attorney writing in the *Washington Post* in 2017, “in the past 35 years, the only significant victory in the battle to control drug prices has been the enactment of legislation that established the generic drug program at the FDA [Federal Drug Administration].” Otherwise, he stated, “prescription drug prices account for 17% of the nation’s health-care costs, up from 7% in the 1990s,” and “prescription drug spending accounts for nearly 20% of total program spending for Medicare, the largest of the governmental

health-care programs.” (Schultz is not entirely impartial; he is a partner in a law firm that represents generic drug providers, among other clients.)

The only way to recoup the enormous cost of developing new drugs, says the pharmaceutical industry, is to pass it along to consumers. Critics, on the other hand, assert that much of the expense incurred within the industry results from the high cost of marketing new drugs. Wherever the truth lies in this debate, it remains that exorbitant prices for much-needed medicines dramatically reduce their social value when only some individuals can actually afford to obtain them.

➞ **EXAMPLE** People with deadly allergies, such as to bee stings or nuts, almost always carry EpiPen products, which provide a quick dose of ephedrine that ameliorates the anaphylaxis (inability to breathe) that can come with a severe allergic reaction. In 2007, the company Mylan bought the EpiPen company and immediately quadrupled the price. Why not? It maximizes profits for shareholders, and because at the time there was no competing product, they had the market cornered. Moreover, since people’s lives literally depended on the product, they had to buy it. While there was a severe public outcry and class action suits against the company, the episode brought attention to a critical ethical question about for-profit medicines. What does it say of our priorities if we have the technology to create life-saving medicines but allow astronomic prices effectively to deny them to many patients who require them?

In the early 21st century, “Big Pharma” became one of the biggest targets of political rhetoric, with even conservative candidates promising to get prescription drug prices under control. At the same time, these promises rarely lead to anything tangible. But of course, most people working in the industry feel they are behaving ethically, providing customers with lifesaving or life-altering medicines while returning profits to shareholders and, perhaps, living a good life. And of course, pharmaceutical companies are no different from others in adhering to a code of ethics, though the code may stress legal compliance and safety over any responsibility to make drugs affordable.



TRY IT

Read the code of ethics for Pfizer, a German-owned company, here:

www.pfizer.com/about/responsibility/compliance/code-of-conduct

You might also seek out more codes of ethics from health providers and pharmaceutical companies.

3. The Affordable Care Act

Health care reform on a major scale emerged in the United States with the passage of the Patient Protection and Affordable Care Act, more commonly known as the **Affordable Care Act (ACA)**, in March 2010, during the Obama administration. The ACA (so-called **Obamacare**) is a complex set of laws that make insurance attainable for many people and attempts to constrain costs. Opponents brand the plan as “socialist.” For its supporters, however, it is the first effective and comprehensive plan to extend affordable health care to the widest segment of the U.S. population; indeed, the number of people without insurance was cut in half by the plan. Furthermore, like most new federal policies, it has undergone tweaks and revision each year since becoming law. The ACA is funded by a combination of payments by enrollees and supplemental federal monies earmarked for this task.

The ethical debate over universal health care coverage is larger even than a business and its employees, of course, but it still carries immense consequences for management and labor irrespective of how the ACA or other legislation fares in the halls of government and the courts. An ethical dilemma for employers is the extent to which they should make health coverage available to their workers at affordable rates, particularly if

federal and state government plans provide little or no coverage for residents and the costs of employer-provided coverage continues to climb.



THINK ABOUT IT

Is free or inexpensive access to health care a basic human right? If so, which elements within society bear the principal responsibility for providing it: government, business, workers, all these, or other agencies or individuals? This is a foundational ethical question that would invoke different responses on the part of nearly everyone you may ask.

As a nation, the United States has usually preferred a system predicated on private health care providers and insurers to pay for it. This arrangement has worked best in instituting high-quality care with minimal delays even for elective medical procedures. It has systematically failed, however, in establishing any sort of universal dispensation that is affordable for many citizens.

Yet the United States is moving slowly and with plenty of hiccups toward some degree of national or state management of health care. Precisely where these efforts will take us may not be clear for the next several years. The political, economic, and ethical dimensions of public management of our health care drive considerable controversy and very little agreement.

In the meantime, difficult ethical dilemmas exist for hospital administrators, who must grapple with keeping costs affordable and keeping the hospital solvent, if not profitable, to investors. It is also a challenge for employers as they are expected to provide health insurance to all workers, and costs continue to rise. Many pass on costs to their employees, or make different options available where some cheaper policies only provide coverage for catastrophe and others provide more coverage for routine doctor visits and out-of-pocket expenses, but at considerable cost to the employee, particularly those with dependents.

One advantage of a single-payer health care system to businesses outside the health industry is that they would be free of these obligations and could even give employees a raise with the money saved from insurance.



TERM TO KNOW

The Affordable Care Act (Obamacare)

A complex set of laws that make insurance attainable for many people and attempts to constrain costs.



SUMMARY

In this lesson, you learned about **single-payer vs. multi-payer health care** systems, where the costs of health care are paid by either a single source, like the government, or a combination of patients, employers, insurance companies, government, and private charities. Access to quality health care is a serious issue with many ethical dimensions, but universal health care systems also present ethical challenges, from the redistribution of resources to long wait times for surgeries to the need to ration health care. **Pharmaceuticals** are an important part of modern health care and the need for resources for research and development that comes from a profit is often compared to the limited access to lifesaving medications. Health care, and deciding who gets it, who does not, and who pays for it, is one of the most pressing ethical issues for everyone: policymakers, health care providers, businesses, and families. **The Affordable Care Act**, more popularly known as Obamacare, was one attempt to insure more people and ameliorate the cost.

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